

Divided into subsections:

- 1. Prevention**
- 2. Assessment**
- 3. Infection**
- 4. Wound care**
- 5. Footwear and follow up**
- 6. Charcot's foot**
- 7. Surgical intervention and Revascularization**

Key terms used:

Peripheral Neuropathy: The presence of symptoms or signs of peripheral nerve dysfunction in people with diabetes, after exclusion of other causes.

Peripheral Artery Disease (PAD): Obstructive atherosclerotic vascular disease with clinical symptoms, signs or abnormalities on non-invasive vascular assessment, resulting in disturbed or impaired circulation in one or more extremities.

4. 1: PREVENTION

Overview:

1. Identification of the at-risk foot
2. Regular inspection and examination of the at-risk foot
3. Education of patient, family and healthcare providers
4. Routine wearing of appropriate footwear
5. Treatment of pre-ulcerative signs

4.1.1 Prevention of Diabetic foot problems

4.1.1 To identify a person with diabetes at risk for foot ulceration, examine the feet annually / six monthly / quarterly / monthly (depending on patient's risk category) to seek evidence for signs or symptoms of peripheral neuropathy and peripheral artery disease. IWGDF 2015 (Adapted)

Risk Classification System and preventive screening frequency

Category	Characteristic	Frequency
0	No peripheral neuropathy	Once a year
1	Peripheral neuropathy	Once every 6 months
2	Peripheral neuropathy with peripheral artery disease and/or a foot deformity	Once every 3-6 months

3	Peripheral neuropathy and a history of foot ulcer or lower-extremity amputation	Once every 1-3 months
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source : *The IWGDF guidelines 2015*

4.1.2 In a person with diabetes who has peripheral neuropathy, screen for: a history of foot ulceration or lower-extremity amputation; peripheral artery disease; foot deformity; pre-ulcerative signs on the foot; poor foot hygiene; and ill-fitting or inadequate footwear. (Strong; Low) IWGDF 2015 (Adopted)

4.1.3 Treat any pre-ulcerative sign on the foot of a patient with diabetes. This includes: removing callus; protecting blisters and draining when necessary; treating ingrown or thickened toe nails; treating haemorrhage when necessary; and prescribing antifungal treatment for fungal infections. (Strong; Low) IWGDF 2015 (Adopted)

4.1.4 To protect their feet, instruct an at-risk patient with diabetes not to walk barefoot, in socks, or in thin-soled standard slippers, whether at home or when outside. (Strong; Low) IWGDF 2015 (Adopted)

4.1.5 Instruct an at-risk patient with diabetes to: daily inspect their feet and the inside of their shoes; daily wash their feet (with careful drying particularly between the toes); avoid using chemical agents or plasters to remove callus or corns; Adopted use emollients to lubricate dry skin; and cut toe nails straight across. (Weak; Low) IWGDF 2015 (Adopted)

4.1.6 Instruct an at-risk patient with diabetes to wear properly fitting footwear to prevent a first foot ulcer, either plantar or non-plantar, or a recurrent non-plantar foot ulcer. When a foot deformity or a pre-ulcerative sign is present, consider prescribing therapeutic shoes, custom-made insoles, or toe orthosis. (Strong; Low) IWGDF 2015 (Adopted)

4.1.7 Instruct a high-risk patient with diabetes to monitor foot skin temperature at home to prevent a first or recurrent plantar foot ulcer. This aims at identifying the early signs of inflammation, followed by action taken by the patient and care provider to resolve the cause of inflammation. (Weak; Moderate) IWGDF 2015 (Adopted)

4.1.8 To prevent a first foot ulcer in an at-risk patient with diabetes, provide education aimed at improving foot care knowledge and behaviour, as well as encouraging the patient to adhere to this foot care advice. (Weak; Low) IWGDF 2015 (Adopted)

4.1.9 To prevent a recurrent plantar foot ulcer in an at-risk patient with diabetes, prescribe therapeutic footwear that has a demonstrated plantar pressure relieving effect during walking and encourage the patient to wear this footwear. (IWGDF 2015 (Adapted)

4.1.10 To prevent a recurrent foot ulcer in an at-risk patient with diabetes, provide integrated foot care, which includes professional foot treatment, adequate footwear and education. This should be repeated or re-evaluated once every one to three months as necessary. (Strong; Low) IWGDF 2015 (Adopted)

4.2.1: Assessing the risk of developing a diabetic foot problem